



AI Playbook and Toolkit for Public Health Departments

Organizational Change Management for AI Adoption

EXE 430

AI adoption changes how people work, make decisions, document actions, communicate with partners, and understand their roles. Successful implementation requires change management, not just technical deployment. Staff need to understand why a tool is being introduced, how it affects their workflow, what they are responsible for reviewing, and how concerns will be handled. This module helps leaders plan the organizational side of AI adoption. It focuses on stakeholder engagement, communication, training, workflow redesign, trust-building, feedback loops, readiness assessment, and adoption monitoring. The goal is to prevent AI from being experienced as a top-down technology mandate and instead support thoughtful integration into public health practice.

Level	advanced/leadership
Primary track	Public Health Executive Leadership Track
Appears in tracks	governance-security

Learning Objectives

- Explain why AI implementation requires change management.
- Identify stakeholders affected by an AI-supported workflow.
- Develop a change management plan that includes communication, training, workflow updates, feedback, and adoption measures.
- Recognize staff concerns related to trust, workload, deskilling, surveillance, accountability, and job impact.
- Produce an AI adoption and change management plan.

Module Overview

AI adoption changes how people work, make decisions, document actions, communicate with partners, and understand their roles. Successful implementation requires change management, not just technical deployment. Staff need to understand why a tool is being introduced, how it affects their workflow, what they are responsible for reviewing, and how concerns will be handled.

This module helps leaders plan the organizational side of AI adoption. It focuses on stakeholder engagement, communication, training, workflow redesign, trust-building, feedback loops, readiness assessment, and adoption monitoring. The goal is to prevent AI from being experienced as a top-down technology mandate and instead support thoughtful integration into public health practice.

Why This Topic Matters for Public Health AI

AI tools may fail even when the technology works because the organization is not ready to use them. Staff may not trust the output, may over-trust it, may not understand their review responsibilities, or may develop workarounds that undermine safeguards. Leaders may underestimate the time needed for training, workflow redesign, and user feedback.

Change management is also an equity and workforce issue. Staff who are not included in design may experience AI as a threat or burden. Programs with less technical capacity may be left behind. Communities may experience changes in service delivery without understanding how AI is involved. A change plan helps ensure that adoption is transparent, inclusive, and aligned with public health values.

Public Health Example

A health department launches an AI assistant to help staff find internal policies and program guidance. The tool is technically functional, but staff do not use it because they are unsure whether answers are official, whether searches are monitored, and whether they remain responsible for verifying information. Some staff worry that the tool will be used to judge their productivity.

A change management plan addresses these concerns directly. It explains the purpose of the tool, approved uses, prohibited uses, privacy protections, source citation requirements, human verification expectations, and feedback process. It also includes supervisors, program champions, training sessions, office hours, and monitoring of adoption barriers.

Technical and Governance Deep Dive

AI change management should begin before procurement or configuration. Leaders should identify who will be affected, what pain points the tool is supposed to address, which workflows will change, and what staff need to learn. Stakeholder engagement should include frontline users, supervisors, program leaders, IT, privacy, legal, communications, equity staff, and where appropriate, community representatives.

The communication plan should be specific. Staff need to know what the tool does, what it does not do, why the agency is using it, what data may be entered, what outputs require review, and how errors should be reported. Communication should also acknowledge uncertainty. Overselling AI can damage trust when limitations become visible. Under-explaining AI can lead to rumors, resistance, or unsafe use.

Training should be tied to workflow. Generic AI literacy is not enough. Users need role-specific examples, review criteria, escalation scenarios, documentation expectations, and practice with realistic outputs. Supervisors need to know how to reinforce appropriate use and respond to concerns. Change management should continue after launch through feedback, monitoring, and iterative improvement.

Risks, Failure Modes, and Guardrails

One risk is adoption without understanding. Staff may use the tool incorrectly or treat outputs as authoritative because training was superficial. A guardrail is to require role-based training and workflow-specific practice before access is granted.

Another risk is resistance caused by lack of transparency. If staff believe AI is being introduced to monitor or replace them, they may avoid or undermine the tool. A guardrail is to communicate purpose, boundaries, data practices, and workforce implications clearly and early.

Application to AI-Supported Workflows

Generative AI adoption requires training on review, citation, privacy, and public records. Predictive analytics adoption requires training on interpretation, limitations, and override documentation. Agentic workflow adoption requires training on approvals, stop procedures, and escalation. Each AI type changes work in different ways, so change management must be specific.

Reflection Questions

Which current or proposed AI use case in your agency raises this leadership or governance issue most clearly?

Who currently has authority to make decisions about this issue, and is that authority documented?

What evidence would governance or leadership need before approving the use case?

Which staff, partners, or communities would be affected if this issue were handled poorly?

What policy, process, or artifact should be created or updated after this module?

Practical Exercise

Create a change management plan for one AI use case. Include stakeholder map, communication messages, training needs, workflow changes, adoption measures, feedback process, and launch support.

Before You Begin

Choose a real or realistic public health AI use case. Document assumptions, unresolved questions, and which agency roles would need to review the artifact before it is adopted. The exercise should produce a work product that could support governance review, leadership discussion, implementation planning, or policy development.

Expected Artifact or Evidence

AI adoption plan; stakeholder map; communication plan; training plan; feedback and adoption monitoring plan.

Expected Assignment

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- AI adoption plan; stakeholder map; communication plan; training plan; feedback and adoption monitoring plan.

Knowledge Check

- 1. What is the strongest reason this module topic belongs in AI leadership and governance training?
- 2. Which practice best supports accountable public health AI governance?
- 3. When should an AI use case be escalated for leadership or governance review?
- 4. What is weak evidence of completion for a leadership and governance module?
- 5. What should leaders do when an AI use case has meaningful benefits but unresolved risks?

References and Resources

- NIST Artificial Intelligence Risk Management Framework (AI RMF 1.0):
<https://www.nist.gov/itl/ai-risk-management-framework>
- NIST Generative AI Profile: <https://www.nist.gov/publications/artificial-intelligence-risk-management-framework-generative-artificial-intelligence>
- NIST Privacy Framework
- CDC Public Health Data Strategy and Data Modernization resources:
<https://www.cdc.gov/public-health-data-strategy/php/about/index.html>
- WHO Ethics and Governance of Artificial Intelligence for Health:
<https://www.who.int/publications/i/item/9789240029200>
- OMB Memorandum M-24-10 on federal AI governance, risk management, and innovation, where relevant to public-sector governance models
- Agency strategic plans, procurement policies, privacy policies, cybersecurity policies, records policies, and equity plans